

|                                     |                                                   |
|-------------------------------------|---------------------------------------------------|
| <b>Patient Name</b> : Mr. KAIF ALI  | <b>Specimen Drawn ON</b> : 04/Feb/2026 10:00AM    |
| <b>Age/Gender</b> : 19 YRS /M       | <b>Specimen Received ON</b> : 04/Feb/2026 07:00PM |
| <b>UHID/MR No</b> : ADEL.0004092159 | <b>Report Date</b> : 04/Feb/2026 07:30PM          |
| <b>Barcode No</b> : D3062115        | <b>Client Code</b> : DL2838                       |
| <b>Ref Doctor</b> : Dr.SELF         | <b>Visit ID</b> : MDEL4095247                     |
| <b>Ref Customer</b> : SELF          | <b>Client Name</b> : LIFE CARE DIAGNOSTIC CENTRE  |

**DEPARTMENT OF HAEMATOLOGY**

| Test Name | Result | Unit | Bio. Ref. Range | Method |
|-----------|--------|------|-----------------|--------|
|-----------|--------|------|-----------------|--------|

**COMPLETE BLOOD COUNT(CBC)23**

|                        |              |               |            |                        |
|------------------------|--------------|---------------|------------|------------------------|
| R.B.C                  | 4.94         | Millions/cumm | 4.5-5.5    | Impedance variation    |
| <b>Haemoglobin</b>     | 14.5         | g/dl          | 13-17      | Spectrophotometry      |
| Packed Cell Volume     | 44.50        | %             | 40.0-50.0  | Analogical Integration |
| MCV                    | 90.08        | fL            | 80-100     |                        |
| MCH                    | 29.35        | pg            | 27.0-32.0  | Calculated             |
| MCHC                   | 32.58        | g/dL          | 27.0-48.0  | Calculated             |
| RDW-CV                 | <b>15.8</b>  | %             | 11.5-14.0  | Calculated             |
| <b>Platelet Count</b>  | 195          | x1000/uL      | 150-450    | Impedance Variation    |
| <b>Total WBC Count</b> | 4670         | /cumm         | 4000-10000 | Impedance Variation    |
| TNC                    | 4.67         |               |            |                        |
| MPV                    | 10.40        | %             | 9.1-11.9   | Calculated             |
| PCT                    | 0.20         | %             | 0.18-0.39  | Calculated             |
| PDW                    | <b>19.60</b> | %             | 9.0-15.0   | Calculated             |

**Differential Leucocyte Count**

|                             |      |         |           |                       |
|-----------------------------|------|---------|-----------|-----------------------|
| Neutrophil                  | 50   | %       | 40.0-80.0 | flow cytometry/manual |
| Lymphocyte                  | 40   | %       | 20.0-40.0 | flow cytometry/manual |
| Monocytes                   | 08   | %       | 2-10      | flow cytometry/manual |
| Eosinophils                 | 02   | %       | 01-06     | Flow cytometry/manual |
| Basophils                   | 00   | %       | 0-2       | Flow cytometry/manual |
| Absolute Neutrophils        | 2.34 | 1000/uL | 2.00-7.00 |                       |
| Absolute Lymphocytes        | 1.87 | 1000/μL | 1.00-3.00 |                       |
| Absolute Monocytes          | 0.37 | 1000/μL | 0.20-1.00 |                       |
| Absolute Eosinophils        | 0.09 | 1000/μL | 0.02-0.50 |                       |
| Neutrophil-Lymphocyte Ratio | 1.25 |         |           | Calculated            |
| Lymphocyte-Monocyte Ratio   | 5    |         |           | Calculated            |
| Platelet-Lymphocyte Ratio   | 5    |         |           | Calculated            |

This report has been validated by:



DR. NEERU AGARWAL  
M.B.B.S., M.D. (PATH)  
LAB HEAD  
REGD NO. 21087



DR. ANIL GUPTA  
M.B.B.S., M.D. (PATH)  
SR. CONSULTANT PATHOLOGIST  
REGD. NO. 5015

DR. SMRITHI KRISHNA C  
M.B.B.S., M.D. (PATH)  
CONSULTANT PATHOLOGIST  
REGD NO. 83525

DR. SONAM  
M.B.B.S., M.D. (PATH)  
SR. CONSULTANT PATHOLOGIST  
REGD. NO. 4268



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**CRL DIAGNOSTICS PVT. LTD.**

Plot No. 10, Avtar Enclave, Opposite Metro Pillar No. 227, Paschim Vihar, Rohtak Road, New Delhi-110063  
E-mail : info@crl diagnostics.com, web : www.crl diagnostics.com, Phone : + 011-42-78-78-78

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| <b>UHID/MR No</b> : ADEL.0004092159 | <b>Report Date</b> : 04/Feb/2026 07:54PM          |
| <b>Barcode No</b> : D3062115        | <b>Client Code</b> : DL2838                       |
| <b>Ref Doctor</b> : Dr.SELF         | <b>Visit ID</b> : MDEL4095247                     |
| <b>Ref Customer</b> : SELF          | <b>Client Name</b> : LIFE CARE DIAGNOSTIC CENTRE  |

| DEPARTMENT OF HAEMATOLOGY |        |      |                 |        |
|---------------------------|--------|------|-----------------|--------|
| Test Name                 | Result | Unit | Bio. Ref. Range | Method |

| MALARIAL PARASITE SPECIES IDENTIFICATION |          |  |  |                                |
|------------------------------------------|----------|--|--|--------------------------------|
| Malarial Parasite Identification         | NEGATIVE |  |  | Leishman Staining & Microscopy |




DR. NEERU AGARWAL  
M.B.B.S., M.D. (PATH)  
LAB HEAD  
REGD NO. 21087



DR. ANIL GUPTA  
M.B.B.S., M.D. (PATH)  
SR. CONSULTANT PATHOLOGIST  
REGD. NO. 5015

DR. SMRITHI KRISHNA C  
M.B.B.S., M.D. (PATH)  
CONSULTANT PATHOLOGIST  
REGD NO. 83525

This report has been validated by:

DR. SONAM  
M.B.B.S., M.D. (PATH)  
SR. CONSULTANT PATHOLOGIST  
REGD. NO. 4268



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|                                     |                                                   |
|-------------------------------------|---------------------------------------------------|
| <b>Patient Name</b> : Mr. KAIF ALI  | <b>Specimen Drawn ON</b> : 04/Feb/2026 10:00AM    |
| <b>Age/Gender</b> : 19 YRS /M       | <b>Specimen Received ON</b> : 04/Feb/2026 06:59PM |
| <b>UHID/MR No</b> : ADEL.0004092159 | <b>Report Date</b> : 04/Feb/2026 08:12PM          |
| <b>Barcode No</b> : D3062116        | <b>Client Code</b> : DL2838                       |
| <b>Ref Doctor</b> : Dr.SELF         | <b>Visit ID</b> : MDEL4095247                     |
| <b>Ref Customer</b> : SELF          | <b>Client Name</b> : LIFE CARE DIAGNOSTIC CENTRE  |

**DEPARTMENT OF BIOCHEMISTRY**

| Test Name | Result | Unit | Bio. Ref. Range | Method |
|-----------|--------|------|-----------------|--------|
|-----------|--------|------|-----------------|--------|

**LIVER FUNCTION TEST (LFT)-EXTENDED**

**Sample Type : SERUM**

|                                         |             |       |           |                                                     |
|-----------------------------------------|-------------|-------|-----------|-----------------------------------------------------|
| <b>Bilirubin Total</b>                  | 0.73        | mg/dl | <1.1      | Diazotized Sulfanilic                               |
| <b>Bilirubin Direct</b>                 | 0.19        | mg/dl | 0-0.3     | Diazotized Sulfanilic                               |
| <b>Bilirubin Indirect</b>               | 0.54        | mg/dl | 0.30-1.00 | Calculated                                          |
| <b>SGOT (AST)</b>                       | <b>34.5</b> | U/L   | <31.0     | IFCC without pyridoxal phosphate                    |
| <b>SGPT (ALT)</b>                       | 29.4        | U/L   | <33.0     | IFCC without pyridoxal phosphate                    |
| <b>Alkaline Phosphatase (ALP)</b>       | 93.6        | U/L   | 55-149    | Spectrophotometry                                   |
| <b>Gamma Glutamyl Transferase (GGT)</b> | <b>14.1</b> | U/L   | 15-60     | L-Gamma-glutamyl-3-carboxy-4-nitroanilide Substrate |
| <b>Protein Total</b>                    | 7.5         | g/dL  | 6.6-8.7   | Biuret                                              |
| <b>Albumin (Serum)</b>                  | <b>5.09</b> | g/dL  | 3.2-4.5   | Bromo Cresol Green (BCG)                            |
| <b>Globulin</b>                         | <b>2.41</b> | g/dL  | 2.50-3.50 | Calculated                                          |
| <b>A/G Ratio</b>                        | 2.11        |       | 1.5-2.5   | Calculated                                          |

**Interpretation:-** Liver blood tests, or liver function tests, are used to detect and diagnose disease or inflammation of the liver. Elevated aminotransferase (ALT, AST) levels are measured as well as alkaline phosphatase, albumin, and bilirubin. Some diseases that cause abnormal levels of ALT and AST include hepatitis A, B, and C, cirrhosis, iron overload, and Tylenol liver damage. Medications also cause elevated liver enzymes. There are less common conditions and diseases that also cause elevated liver enzyme levels.: Liver blood tests, or liver function tests, are used to detect and diagnose disease or inflammation of the liver. Elevated aminotransferase (ALT, AST) levels are measured as well as alkaline phosphatase, albumin, and bilirubin. Some diseases that cause abnormal levels of ALT and AST include hepatitis A, B, and C, cirrhosis, iron overload, and Tylenol liver damage. Medications also cause elevated liver enzymes. There are less common conditions and diseases that also cause elevated liver enzyme levels.

*Pawan Kumar*  
DR. PAWAN KUMAR  
Phd. BIOCHEMISTRY  
CONSULTANT BIOCHEMIST

*Anil Gupta*  
DR. ANIL GUPTA  
M.B.B.S., M.D. (PATH)  
SR. CONSULTANT PATHOLOGIST  
REGD. NO. 5015

This report has been validated by:  
*Neeru Agarwal*  
DR. NEERU AGARWAL  
M.B.B.S., M.D. (PATH)  
LAB HEAD  
REGD NO. 21087



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| <b>Age/Gender</b> : 19 YRS /M       | <b>Specimen Received ON</b> : 04/Feb/2026 06:59PM |
| <b>UHID/MR No</b> : ADEL.0004092159 | <b>Report Date</b> : 04/Feb/2026 08:25PM          |
| <b>Barcode No</b> : D3062116        | <b>Client Code</b> : DL2838                       |
| <b>Ref Doctor</b> : Dr.SELF         | <b>Visit ID</b> : MDEL4095247                     |
| <b>Ref Customer</b> : SELF          | <b>Client Name</b> : LIFE CARE DIAGNOSTIC CENTRE  |

DEPARTMENT OF IMMUNOASSAY

| Test Name | Result | Unit | Bio. Ref. Range | Method |
|-----------|--------|------|-----------------|--------|
|-----------|--------|------|-----------------|--------|

THYROID PROFILE

Sample Type : SERUM

|                                    |       |        |           |                                      |
|------------------------------------|-------|--------|-----------|--------------------------------------|
| <b>Triiodothyronine Total (T3)</b> | 1.05  | ng/mL  | 0.80-2.10 | Chemiluminescence Immunoassay (CLIA) |
| <b>Thyroxine Total (T4)</b>        | 7.33  | ug/dL  | 4.6-10.5  | Chemiluminescence Immunoassay (CLIA) |
| <b>TSH (4th Generation)</b>        | 2.700 | uIU/mL | 0.70-6.40 | Chemiluminescence Immunoassay (CLIA) |

| PREGNANCY     | REFERENCE RANGE for TSH IN uIU/mL (As per American Thyroid Association.) |
|---------------|--------------------------------------------------------------------------|
| 1st Trimester | 0.10-2.50 uIU/mL                                                         |
| 2nd Trimester | 0.20-3.00 uIU/mL                                                         |
| 3rd Trimester | 0.30-3.00 uIU/mL                                                         |

**INTERPRETATION-**

1. Primary hyperthyroidism is accompanied by elevated serum T3 & T4 values along with depressed TSH level.
2. Primary hypothyroidism is accompanied by depressed serum T3 and T4 values & elevated serum TSH levels.
3. Normal T4 levels accompanied by high T3 levels and low TSH are seen in patients with T3 thyrotoxicosis.
4. Normal or low T3 & high T4 levels indicate T4 thyrotoxicosis (problem is conversion of T4 to T3)
5. Normal T3 & T4 along with low TSH indicate mild / subclinical HYPERTHYROIDISM .
6. Normal T3 & low T4 along with high TSH is seen in HYPOTHYROIDISM .
7. Normal T3 & T4 levels with high TSH indicate Mild / Subclinical HYPOTHYROIDISM .
8. Slightly elevated T3 levels may be found in pregnancy and in estrogen therapy while depressed levels may be encountered in severe illness , malnutrition , renal failure and during therapy with drugs like propranolol.
9. Although elevated TSH levels are nearly always indicative of primary hypothyroidism . rarely they can result from TSH secreting pituitary tumours ( secondary hyperthyroidism )

\*TSH IS DONE BY ULTRASENSITIVE 4th GENERATION CHEMIFLEX ASSAY\*

**COMMENTS:**

Assay results should be interpreted in context to the clinical condition and associated results of other investigations. Previous treatment with corticosteroid therapy may result in lower TSH levels while thyroid hormone levels are normal. Note:

TSH levels may fluctuate based on few factors such as pregnancy, illness and age. Also, time of sample collection, technologies used to analyze the test, usage of certain drugs. Diet may have impact on TSH levels. TSH may show around 50% variation even when done at different times of day due to its association with circadian rhythm.

*Pawan Kumar*

DR. PAWAN KUMAR  
Phd. BIOCHEMISTRY  
CONSULTANT BIOCHEMIST

*Anil Gupta*

DR. ANIL GUPTA  
M.B.B.S., M.D. (PATH)  
SR. CONSULTANT PATHOLOGIST  
REGD. NO. 5015

This report has been validated by:

*Neeru Agarwal*

DR. NEERU AGARWAL  
M.B.B.S., M.D. (PATH)  
LAB HEAD  
REGD NO. 21087



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**Laboratory Test Report**

|                                     |                                                   |
|-------------------------------------|---------------------------------------------------|
| <b>Patient Name</b> : Mr. KAIF ALI  | <b>Specimen Drawn ON</b> : 04/Feb/2026 10:00AM    |
| <b>Age/Gender</b> : 19 YRS /M       | <b>Specimen Received ON</b> : 04/Feb/2026 06:59PM |
| <b>UHID/MR No</b> : ADEL.0004092159 | <b>Report Date</b> : 04/Feb/2026 10:43PM          |
| <b>Barcode No</b> : D3062116        | <b>Client Code</b> : DL2838                       |
| <b>Ref Doctor</b> : Dr.SELF         | <b>Visit ID</b> : MDEL4095247                     |
| <b>Ref Customer</b> : SELF          | <b>Client Name</b> : LIFE CARE DIAGNOSTIC CENTRE  |

| DEPARTMENT OF SEROLOGY |        |      |                 |        |
|------------------------|--------|------|-----------------|--------|
| Test Name              | Result | Unit | Bio. Ref. Range | Method |

|                     |      |       |     |      |
|---------------------|------|-------|-----|------|
| HIV I & II Antibody | 0.01 | AU/mL | 0-1 | CLIA |
|---------------------|------|-------|-----|------|

COMMENTS: Non reactive results indicate that antibodies to HIV I/II have not been detected in the sample. This implies either non exposure to HIV I/II or that the individual might be in the 'Window Period', that is, prior to the development of detectable level of Antibodies. Therefore, a non-reactive result does not exclude the possibility of a persisting HIV infection. Reactive samples must be confirmed by using HIV Western Blot as some degree of cross reactivity for HIV antibodies has been noted with certain other naturally occurring antibodies and also antibodies formed in response to other bacterial and viral infections, and in certain autoimmune disorders. Post test counseling will be done by the concerned referring doctor. The sensitivity and specificity of this test has been determined by National HIV Reference Centers of Govt.of India and WHO collaborating center, using various other test panels.

This report has been validated by:



DR. SWATI JAIN  
M.B.B.S , M.D. (MICRO)  
CONSULTANT MICROBIOLOGIST  
DMC NO. 61188

DR. ANIL GUPTA  
M.B.B.S , M.D. (PATH)  
SR. CONSULTANT PATHOLOGIST  
REGD. NO. 5015



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**CRL DIAGNOSTICS PVT. LTD.**

Plot No. 10, Avtar Enclave, Opposite Metro Pillar No. 227, Paschim Vihar, Rohtak Road, New Delhi-110063  
E-mail : [info@crliagnostics.com](mailto:info@crliagnostics.com), web : [www.crliagnostics.com](http://www.crliagnostics.com), Phone : + 011-42-78-78-78

|                                     |                                                   |
|-------------------------------------|---------------------------------------------------|
| <b>Patient Name</b> : Mr. KAIF ALI  | <b>Specimen Drawn ON</b> : 04/Feb/2026 10:00AM    |
| <b>Age/Gender</b> : 19 YRS /M       | <b>Specimen Received ON</b> : 04/Feb/2026 06:59PM |
| <b>UHID/MR No</b> : ADEL.0004092159 | <b>Report Date</b> : 05/Feb/2026 12:52AM          |
| <b>Barcode No</b> : D3062116        | <b>Client Code</b> : DL2838                       |
| <b>Ref Doctor</b> : Dr.SELF         | <b>Visit ID</b> : MDEL4095247                     |
| <b>Ref Customer</b> : SELF          | <b>Client Name</b> : LIFE CARE DIAGNOSTIC CENTRE  |

| DEPARTMENT OF SEROLOGY |        |      |                 |        |
|------------------------|--------|------|-----------------|--------|
| Test Name              | Result | Unit | Bio. Ref. Range | Method |

**WIDAL (SLIDE AGGLUTINATION)**

**Sample Type : SERUM**

|                           |       |  |                |               |
|---------------------------|-------|--|----------------|---------------|
| S. Typhi Antigen (O)      | <1:80 |  | <1:80 Negative | Agglutination |
| S. Typhi Antigen (H)      | <1:80 |  | <1:80 Negative | Agglutination |
| S. Paratyphi Antigen (AH) | <1:80 |  | <1:80 Negative | Agglutination |
| S. Paratyphi Antigen (BH) | <1:80 |  | <1:80 Negative | Agglutination |

**COMMENT:**

Significant titer: For 'O' Antigen- agglutination titer of >1/80 is considered significant.

For 'H' Antigen- agglutination titer of >1/80 is considered significant.

In Salmonella Typhi infection- Antibodies to TO & TH antigens are raised.

In Salmonella Paratyphi A infection- Antibodies to TO & AH antigens are raised.

In Salmonella Paratyphi B infection- Antibodies to TO & BH antigens are raised.

**NOTE:**

Widal test is indicated in 2-3 weeks of illness. It is a slide agglutination test that detects antibodies in the patient's serum against antigens of S. Typhi & S. Paratyphi. Reactive results indicate recent or ongoing infection by Salmonella species & the diagnosis should be confirmed by Blood culture which is the gold standard test. A single positive test result has less significance, therefore repeat testing is recommended after 1 week to demonstrate rise in titer which is considered as definitive evidence of infection.

False negative results may be seen due to following reasons: very early stage (1<sup>st</sup> week) of illness, antibiotic therapy or prozone phenomena.

False positive results may be seen due to following reasons: prior immunization or unrelated infections.

\*\*\* End Of Report \*\*\*

This report has been validated by:

  
DR. SWATI JAIN  
M.B.B.S., M.D. (MICRO)  
CONSULTANT MICROBIOLOGIST  
DMC NO. 61188

DR. ANIL GUPTA  
M.B.B.S., M.D. (PATH)  
SR. CONSULTANT PATHOLOGIST  
REGD. NO. 5015



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**CRL DIAGNOSTICS PVT. LTD.**

Plot No. 10, Avtar Enclave, Opposite Metro Pillar No. 227, Paschim Vihar, Rohtak Road, New Delhi-110063  
E-mail : info@crl diagnostics.com, web : www.crl diagnostics.com, Phone : + 011-42-78-78-78